



SURGICAL PATHOLOGY REPORT

De-Identified Specimen Code _____

Patient Age/Sex / M

SPECIMEN SUBMITTED:

- Part A: LEFT DISTAL URETER
- Part B: LEFT PELVIC LYMPH NODE
- Part C: RIGHT DISTAL URETER
- Part D: FINAL URETHRAL MARGIN
- Part E: BLADDER AND PROSTATE
- Part F: LEFT DISTAL URETER #2
- Part G: RIGHT PELVIC LYMPH NODES
- Part H: RIGHT COMMON LYMPH NODES
- Part I: LEFT COMMON LYMPH NODES
- Part J: LEFT PELVIC LYMPH NODES
- Part K: PRESACRAL LYMPH NODES

100-0-3

Carcinoma, urothelial, NOS 8/20/3
Site: bladder, NOS C67.9
12/15/11

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Malignancy		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	12/15/11	12/15/11

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Final Diagnosis

1. Left distal ureter, biopsy (A) - Atypical urothelial cells.
 2. Left pelvic lymph nodes, excision (B) - Metastatic urothelial carcinoma involving one lymph node.
 3. Right distal ureter and final urethral margin, biopsies (C, D) - Negative for neoplasm.
 4. Bladder, excision (E) - Invasive poorly differentiated urothelial carcinoma with invasion through muscular wall and involvement of perivesical adipose tissue.
 5. Prostate, excision (E) - Urothelial carcinoma involving prostatic urethra with invasion into adjacent prostatic parenchyma.
 6. Left distal ureter #2, biopsy (F) - Negative for neoplasm.
 7. Right pelvic lymph nodes, excision (G) - Negative for neoplasm, fourteen lymph nodes.
 8. Right common lymph nodes, excision (H) - Negative for neoplasm, nine lymph nodes.
 9. Left common lymph nodes, excision (I) - Metastatic urothelial carcinoma involving two of six lymph nodes.
 10. Left common lymph nodes, excision (J) - Metastatic urothelial carcinoma involving two of four lymph nodes.
 11. Presacral lymph nodes, excision (K) - Negative for neoplasm, five lymph nodes.
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Diagnosis Comment:

2. The carcinoma was only present in the permanent section and not present in the frozen section.

4. Specimen Type: Radical cystoprostatectomy

Tumor Size, Greatest dimension: 3.5 cm

Histologic Type: Urothelial carcinoma

Urothelial Carcinoma: (WHO/ISUP, 1998): High grade

Adenocarcinoma and Squamous Carcinoma: Not applicable

Associated Epithelial Lesions: None identified

Margins: Margins uninvolved by invasive carcinoma

Venous/Lymphatic (Large/Small Vessel) Invasion (V/L): Present

Direct Extension of Invasive Tumor: Perivesical fat
Prostatic stroma

Other Diagnoses: See above

Pathologic Stage:

Primary Tumor: (pT)

pT4a: Tumor invades prostate

Regional Lymph Nodes: (pN)

pN3: Metastasis in common iliac lymph nodes The size of the largest metastasis is 2.2 cm. The tumor extends beyond the lymph node capsules to involve the adjacent adipose tissue.

Distant Metastasis (pM): pMX: Cannot be assessed

Clinical Diagnosis:

BLADDER CA

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Gross Description:

A. Received fresh at the frozen desk is a specimen labeled "left distal ureter" consisting of one piece of unoriented renal tissue measuring 0.8 x 0.6 x 0.3 cm. The specimen is totally submitted for frozen section as A1.

B. Received fresh at the frozen desk is a specimen labeled "left pelvic lymph node" consisting of one ovoid nodule measuring 1.5 x 1 x 0.5 cm. The specimen is totally submitted for frozen section as B1.

C. Received fresh at the frozen desk is a specimen labeled "right distal ureter" consisting of one piece of unoriented luminal tissue measuring 0.7 x 0.7 x 0.3 cm. The specimen is totally submitted for frozen section as C1.

D. Received fresh at the frozen desk is a specimen labeled "final urethral margin" consisting of one piece of unoriented tissue measuring 0.5 x 0.5 x 0.5 cm. The specimen is totally submitted for frozen section in D1.

[REDACTED]

E. Received in formalin previously opened and inked with the margins taken is a specimen which consists of a urinary bladder, bilateral ureters, prostate and attached seminal vesicles and vas deferens. The bladder measures 13.0 x 9.0 x 5.5 cm. The external surface of the bladder is covered by a serosal lining at the dome and fibroadipose tissue at the remaining portion. After the bladder is opened an irregular, tan-white, firm area is identified measuring 3.5 x 3.0 cm. The mass appears to encompass the entire trigone, the anterior, posterior, left bladder walls and both ureteral orifices. The mass is semisolid with tan-brown to white, friable possible papillary areas. On sectioning, the mass appears to extend through the left wall for a distance of 1.5 cm and is 0.2 cm from the soft tissue line of resection (inked black). It does not appear to involve the perivesicular adipose tissue. Purulent material is present upon sectioning of the left wall and left trigone. The remainder of the bladder wall appears edematous and diffusely hemorrhagic. The hemorrhagic areas are predominantly visible on the posterior bladder wall. A 3.0 cm segment of right ureter is present. It is probe patent. Its mucosa is slightly inflamed and granular. A 2.0 cm segment of left ureter is present. It is probe patent and its mucosa is inflamed and granular. The prostate measures 3.0 cm anteroposteriorly, 3.2 cm craniocaudally, and 4.0 cm transversely. The capsular surface appears slightly roughened but complete. The gland is inked blue on the right and yellow on the left, and sectioned from the apex to base transversely at 3 mm intervals perpendicular to the urethra. The cross sections reveal a yellow-tan mottled cut surface. A definitive mass is not identified. Rubbery nodularity is present. The seminal vesicles appear unremarkable. Segments of vas deferens appear unremarkable. Sectioning of the attached fibroadipose tissue reveals three tan-white firm nodules, resembling lymph nodes ranging in size from 0.3-0.6 cm in greatest dimension. Representative sections are submitted in formalin as follows: E1 right distal ureter shave, E2 left distal ureter shave, E3-E4 anterior bladder wall, E5 left wall to include deepest extent of lesion, E6 left ureteral orifice, E7-E8 left trigone, E9 posterior wall demonstrating hemorrhagic mucosa, E10 dome demonstrating hemorrhagic mucosa, E11 right ureteral orifice, E12 right wall, E13 right trigone, E14 right ureteral mucosa (1.0 cm from distal margin), E15 left ureter mucosa (1.2 cm from distal margin), E16 lymph nodes (3), E17 right apex, E18 left apex, E19 3 mm right, E20 3 mm left, E21 15 mm right anterior (capsular surface incomplete), E22 15 mm right posterior to include seminal vesicles, E23 left anterior (capsular surface incomplete), E24 15 mm left posterior to include seminal vesicles.

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Gross Description:

F. Received fresh at the frozen desk is a specimen labeled "left distal ureter #2" consisting of one piece of unoriented luminal tissue measuring 1 x 0.5 x 0.5 cm. The specimen is totally submitted for frozen section in F1.

G. Received in formalin labeled "right pelvic lymph nodes" is a specimen consisting of multiple fragments of fibrofatty tissue aggregating to 7 x 5 x 1.7 cm. On sectioning, 14 possible lymph nodes are identified ranging in diameter from 0.2 to 1 cm. Sections are submitted as follows: G1 five possible lymph nodes, G2 four possible lymph nodes, G3 four possible lymph nodes, G4 one possible lymph node bisected.

H. Received in formalin labeled "right common lymph nodes" is a specimen consisting of multiple fragments of fibrofatty tissue aggregating to 3 x 2 x 1 cm. Nine possible lymph nodes are identified ranging in diameter from 0.2 to 1.5 cm. Sections are submitted as follows: H1 four possible lymph nodes, H2 four possible lymph nodes, H3 one possible lymph node sectioned and totally submitted.

I. Received in formalin labeled "left common lymph node" is a specimen consisting of multiple fragments of fibrofatty tissue aggregating to 9 x 4 x 2 cm. Four possible tan lymph nodes are identified, two large lymph nodes measuring 3 and 4.5 cm in diameter. Sections are submitted as follows: I1 four possible lymph nodes, I2-I4 one lymph node sectioned and totally submitted, I5-I8 one lymph node sectioned and totally submitted.

J. Received in formalin labeled "left pelvic lymph nodes" is a specimen consisting of multiple fragments of fibrofatty tissue aggregating to 6 x 2.5 x 1.5 cm. Three possible lymph nodes are identified ranging in diameter from 0.5 to 3 cm. Also present is an elongated firm cord-like structure measuring 5 x 0.5 x 0.4 cm. Sections are submitted as follows: J1 one lymph node sectioned and totally submitted, J2 one lymph node, J3 cord-like structure sectioned and totally submitted, J4-J5 one lymph node sectioned and totally submitted.

K. Received in formalin labeled "presacral lymph nodes" is a specimen consisting of multiple fragments of fibrofatty tissue aggregating to 3.5 x 2.5 x 0.5 cm. Five possible lymph nodes are identified ranging in diameter from 0.1 to 0.6 cm. The five possible lymph nodes are submitted in K1.

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